

State of Alaska – Office of the Governor — 34th Alaska State Legislature – Second Session
DEPARTMENT REVIEW – PASSED LEGISLATION

BLUE SHEET

Blue Sheets are a department's final bill analysis and provides the Commissioner an opportunity to weigh in on legislation that has passed, with a recommendation to the Governor on action that may be taken– to enact the bill, to veto the bill, or to allow the bill to become Law without signature (LWOS). Much of the information requested in the Blue Sheet can be taken from the department's bill analyses (BA), which were initially submitted at the time the bill was introduced and updated throughout the legislative process.

Blue sheets are due to the GLO three days after the enrolled bill appears on basis, or sooner by request.

Department: DOH

Date: 5/15/2026

Bill Number: HB195

Short Title (as it appears on BASIS): PHARMACIST PRESCRIPTION AUTHORITY

What action do you recommend the Governor take?

Select one.

☒ SIGN ☐ VETO ☐ LWOS

Action Justification – Why do you recommend the above action?

Enabling pharmacists to utilize their full scope of practice is a policy commitment Governor Dunleavy committed Alaska to in the Rural Health Transformation Program (RHTP). Additionally, this bill expands access to primary care; primary care access is essential in reducing overall healthcare costs for the citizens of Alaska.

What does this Bill do?

In addition to providing a succinct bill summary, please also consider the following: Why is this bill needed? How does it specifically change department programs? What is the anticipated impact on the public, the private sector? Does this bill align with the priorities of Governor Dunleavy?

HB 195, as amended by Amendments 1, 2, 3, 4, and 7, significantly revises Alaska statutes relating to pharmacy practice, prescribing authority, controlled substance regulation, and health-care provider terminology. The updated bill expands pharmacist authority, modernizes collaborative practice rules, strengthens opioid-related safeguards, and updates terminology across Alaska statute to change “physician assistant” to “physician associate.”

The bill prohibits boards or the department from regulating or restricting the content, scope, or fees associated with collaborative practice agreements between pharmacists and practitioners.

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Pharmacists may independently provide patient-care services related to general wellness, disease prevention, and minor or self-limiting conditions, including those requiring CLIA-waived diagnostic tests.

The bill seeks to expand the definition of “practitioner” to include pharmacists prescribing or administering controlled substances. It will narrow pharmacist PDMP-registration thresholds to *federal Schedule II–IV* controlled substances instead of all previously listed schedules and revises limits on the types of controlled substances pharmacists may prescribe (e.g., Schedule IA/IIA or Schedule II federal substances), unless used in opioid-use-disorder treatment in a clinic.

The bill adds requirements for pharmacist education in pain management, opioid use, and addiction, and requires at least two hours of related continuing education for license renewal, with exemptions for those who do not prescribe or who lack a DEA registration.

Pharmacists providing patient-care services must adhere to standards of practice, recognize professional limits, and consult or refer to other practitioners when needed. The bill also clarifies definitions of “practice of pharmacy” to include patient-care services such as prescribing or administering medications or devices.

The bill requires *all* major prescriber types—dentists, physicians, APRNs, and optometrists—to offer a prescription for an opioid-overdose reversal drug when issuing an opioid prescription under high-risk conditions (3-day supply, ≥ 50 MME/day, benzodiazepine co-prescription, or history of overdose or SUD).

The bill globally replaces the term “physician assistant” with “physician associate” across numerous statutes, including those related to:

- State Medical Board membership,
- Licensing, prescribing, and telehealth authority,
- Delegation rules,
- Emergency medical authority,
- Death certification,
- Do-not-resuscitate orders,
- Disability certification for DMV IDs,
- Mental health commitment processes,
- Definitions of “health care provider,” “medical professional,” and related terms.

It also updates statutory definitions, requirements, and disciplinary structures to reflect the new terminology and ensures consistency across Alaska’s medical, licensing, emergency response, insurance, and public-safety statutes.

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Detailed Sectional Analysis

The full sectional analysis of the bill should be completed by the lead agency. Other departments should include an analysis for sections of the bill affecting their agencies.

See DCCED’s Blue Sheet for a full analysis. The summary below is only specific to the bill as it relates to the new requirement for prescribers to offer a prescription for a opioid overdose reversal drug if certain conditions are met.

Sec. 1.

Adds “relating to the prescription of opioid overdose drugs” to the bill title.

- Updates the title to reflect the addition of new overdose-prevention requirements.

Sec. 2.

Adds a new paragraph to AS 08.36.355(c) (dentist statutes) defining “opioid overdose drug” by referencing AS 17.20.085(g).

- This creates a consistent definition used throughout the amendment.

Sec. 3.

Adds a new subsection to AS 08.36.355 requiring dentists who prescribe an opioid to also offer the patient a prescription for an opioid overdose drug under any of the following conditions:

- The prescription exceeds a three-day supply
- The daily dosage is 50 MME or more
- The patient is also being prescribed a benzodiazepine
- The patient has a history of overdose or substance-use disorder
- This inserts an overdose-risk mitigation requirement into the dental practice statutes.

Sec. 4.

Adds the same definition of “opioid overdose drug” to AS 08.64.363(c) (physician statutes).

- Aligns physician terminology with the dentist section.

Sec. 5.

Adds a new subsection to AS 08.64.363 requiring physicians to offer an opioid overdose drug when prescribing an opioid under the same four risk-based conditions listed above.

- Ensures overdose-prevention requirements apply to medical doctors as well as dentists.

Sec. 6.

Adds the definition of “opioid overdose drug” to AS 08.68.705(d) (advanced practice registered nurse statutes).

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- Creates consistency across prescriber types.

Sec. 7.

Adds a new subsection to AS 08.68.705 requiring APRNs who prescribe opioids to offer an opioid overdose drug if any of the same four risk factors are present.

- Extends the overdose-prevention policy to the largest group of non-physician primary care prescribers.

Sec. 8.

Adds two new subsections to AS 08.72.276 (optometrists):

(c) Requires optometrists with limited prescribing authority to offer an opioid overdose drug when an opioid prescription meets any of the four criteria.

(d) Incorporates the same definition of “opioid overdose drug” by reference to AS 17.20.085(g).

- Ensures that all professions with opioid-prescribing authority follow the same risk-mitigation rules.

Sec. 9.

Renumbers subsequent bill sections.

- Administrative change required because new sections are added.

Companion or Similar Bills

Please note companion legislation, the sponsor of the companion legislation, and identify any “rider” bills or provisions from other legislation that was included in this final bill. Please include the name of the sponsor of the “rider bill” or provision. If it was added as an amendment in a committee – please indicate the name of the legislator that sponsored the amendment.

- HB 270 Opioid Overdose Drug Prescription by Rep Tomaszewski was rolled in HFIN and remains in the bill
- Amendment language offered by Rep Tomaszewski on behalf of Rep Gray related to changing the names of physician’s assistants to physician’s associates – this was rolled in HFIN and remains in the bill
- SB 147 Pharmacist Scope of Practice by Senator Giessel; companion bill to the original HB 195

Support/Opposition of the Bill

Please identify names, organizations, titles, and phones numbers of those who testified in support or in opposition of this bill.

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Support for bill:

Pam Ventgen, Executive Director, Alaska State Medical Association 907-562-0304
Jennifer Adams, Associate Dean for Academic Programs, Idaho State University 208-373-1868
Nicole Chopski, Executive Officer, Idaho State Boards of Pharmacy 208-334-3233
Anna Brecheisen, PharmD/MBA, Sitka, AK
Erin Frohnappfel, PharmD, PGY2 Critical Care Pharmacy Resident, Cordova, AK
Daniel Nelson, Director of Pharmacy, Chief Andrew Isaac Center Pharmacy 907-451-6682 ext. 3621
Karina Stark, PharmD, Med, Clinical Associate Pharmacy Professor, UAA/ISU L.S. Skaggs College of Pharmacy, Eagle River, AK
Brittany L. Keener, PharmD, ANTHC Ambulatory Pharmacy Manager
Brian McKnight, CPHT-Adv, Pharmacy Systems Analyst, Southcentral Foundation 907-729-3318

Opposition to bill:

Patrick Martin, Alaska Right to Life. 907-753-6397

Note: That while Alaska Right for Life has put in opposition to the initial bill proposal – no additional feedback or updated position has been provided by Right for Life since the House Finance committee adopted additional compromise language that made more explicit scope of practice limitations – to further clarify that pharmacists cannot prescribe mifepristone. To be clear, pharmacists cannot prescribe mifepristone under any version of the bill. Most conservative legislators were satisfied with the compromise but 8 did remain concerned.

Bill Signing ceremony suggestions (if applicable)

Please provide information for consideration for a bill signing ceremony. Information that is helpful includes: a specific conference, event, venue, date, and the names and contact information of stakeholders who should be considered for an invitation to the bill signing ceremony.

Chief Andrew Isaac Center Pharmacy, Fairbanks, AK
Contact: Daniel Nelson, Director of Pharmacy
907-451-6682 ext. 3621

Commissioner's Signature:  _____ Date: 5/26/2026

I have reviewed the above information and provided it to the Governor's Legislative Office for consideration of an action by the Governor on legislation passed during the second session of the 34th Alaska State Legislature.

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